Batterjee Medical College
For Science and Technology

Psychiatry · Clinical Skills

Mental Status Examination

The structured description of a patient's cognitive and emotional functioning at the moment of interview — the psychiatric equivalent of the physical examination.

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Learning Objectives

Mapped to the BMC Knowledge / Skill / Value framework. Prerequisite: psychiatric history taking.

Knowledge & understanding

- State what the MSE is and is not
- List its components in order
- Define mood vs affect, form vs content, and illusion vs hallucination vs pseudohallucination

Skills

- Elicit and describe each component in correct terminology
- Recognise thought-process disturbances and map them to likely conditions
- Classify delusions and hallucination modalities
- Assess judgement and insight and document a complete MSE

Values & attitudes

- Describe what you OBSERVE, not what you infer
- Use precise, non-pejorative language in the record
- Treat the MSE as a repeatable clinical measurement

What the MSE is — and isn't

What it is

- The sum of the examiner's observations and impressions of the patient
- An estimate of overall cognitive and emotional functioning
- A BASELINE for future comparison, and to track progress

What it is NOT

- Not an intelligence test
- Not a detailed memory test
- Not a fully precise measure of cognition, affect and behaviour

Think of it as a SNAPSHOT IN TIME — a cross-section of the mental state during this interview, which is why it is repeated to follow change.

How it's done — and the components

THE MSE — SAME SEQUENCE, EVERY TIME

1	**Appearance, behaviour & attitude** — self-care, activity, eye contact, manner toward you	observed from minute one
2	**Speech** — quantity, rate, quality; the HOW, not the what	observed while talking
3	**Mood & affect** — mood is told (climate); affect is seen (weather)	ask + observe
4	**Thought — form and content** — the wiring vs the message; delusions, suicidal and homicidal ideas	risk screen mandatory
5	**Perception** — illusions, hallucinations by modality, pseudohallucinations	ask directly
6	**Cognition** — consciousness, orientation, attention, memory, abstraction	formal testing
7	**Insight & judgement** — awareness of illness; judge from recent real-life behaviour	history + interview

A fixed order makes omissions obvious — and lets any reader reconstruct the mental state at a glance.

Only cognition and insight need direct testing — the rest is gathered by observation while you take the history.

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PART 1

Observation

Appearance, behaviour and attitude · speech · mood and affect — what you see and hear.

Appearance, behaviour & attitude

Appearance

Body build, self-care, clothing, grooming, hair, nails · facial expression · unusual features such as weight loss.

Behaviour

Level of activity, posture, eye contact · abnormal movements: tics, grimacing, tremor, disinhibition, hallucinatory gestures.

Attitude

Manner toward the examiner, verbal and non-verbal · cooperative, guarded, sarcastic, hostile, seductive, evasive.

Much of the MSE is complete before you ask a single formal question — deliberate observation from the moment you meet the patient.

Speech — describe the how, not the what

Describe by

- QUANTITY — talkative, spontaneous, minimal, poverty of speech
- RATE — fast or pressured versus slow
- QUALITY — volume, flow, tone, coherence, spontaneity, continuity

Impairments

- Stuttering; dysarthria — an articulation problem
- Pressured speech (mania)
- Paucity of speech or increased latency (depression, psychosis)

Speech describes the FORM of talking — rate, volume, flow. The CONTENT of what is said belongs under Thought.

Mood vs. affect

	Mood	Affect
Definition	The sustained, pervasive feeling state — the "climate"	The observed, moment-to-moment emotional expression — the "weather"
Duration	Days to weeks	Seconds to hours
How assessed	SUBJECTIVE — ask the patient (a symptom)	OBJECTIVE — observed by you (a sign)
Changes with stimuli?	Relatively fixed	Shifts with internal or external cues

Mood is what the patient TELLS you; affect is what you SEE. Record mood in the patient's own words where possible.

Affect — the four descriptors

Quality

Euthymic, depressed, anxious, elated, irritable.

Range

Full → restricted → blunted → flat.

Stability

Stable versus labile — rapid shifts.

Congruence

Appropriate versus inappropriate to the thought and mood.

FLAT affect is classically associated with schizophrenia; a LABILE affect suggests mania, intoxication or an organic/frontal cause; INCONGRUENT affect — laughing while describing a tragedy — is a psychotic sign.

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PART 2

Thought & Perception

Thought form versus content · delusions · illusions, hallucinations and pseudohallucinations — the core of descriptive psychopathology.

Thought — form (process) vs. content

Form / process — HOW they think

- Flight of ideas
- Loosening of associations
- Perseveration
- Circumstantiality, tangentiality, thought blocking

Content — WHAT they think

- Delusions
- Obsessions
- Overvalued ideas
- Suicidal and homicidal ideas

Keep the two apart in your write-up: FORM is the wiring; CONTENT is the message.

"The bell fell and shell..."

An elated, over-talkative patient answers your questions in rapid rhyming phrases — "I feel real, the deal is real, the bell will tell".

He jumps between words that sound alike rather than words that make sense.

What is this?

A Neologism

B Clang association

C Circumstantiality

D Thought blocking

Commit to an answer before the next slide.

"The bell fell and shell..."

ANSWER

An elated, over-talkative patient answers your questions in rapid rhyming phrases — "I feel real, the deal is real, the bell will tell".

He jumps between words that sound alike rather than words that make sense.

What is this?

- **A** Neologism
- **B** **Clang association**
- **C** Circumstantiality
- **D** Thought blocking

B — clang association.

Speech linked by sound and rhyme rather than meaning. Together with the elation and pressured speech, it points to mania.

Thought-process disturbances

Term	What it means
Circumstantiality	Over-detailed and indirect — but eventually reaches the point
Tangentiality	Veers off-topic and NEVER returns to the point
Flight of ideas	Rapid topic shifts with some connection still traceable (mania)
Loosening / derailment	Jumps between ideas with little or no logical link
Word salad / incoherence	Completely disorganised, jumbled words
Neologisms	Made-up words, meaningful only to the patient
Clang associations	Links by rhyme and sound, not by meaning
Thought blocking	Sudden interruption of thought and speech mid-stream

Circumstantiality gets there eventually; tangentiality never does. That single contrast is worth memorising.

Which thought disorder → which condition?

Sign	Points toward
Flight of ideas · clang associations	Mania
Loosening of associations · neologisms · word salad · thought blocking	Schizophrenia — formal thought disorder
Circumstantiality	OCD, epilepsy — or normal in the verbose
Tangentiality	Schizophrenia, mania, or organic states
Perseveration	Organic or frontal-lobe pathology, dementia
Poverty of thought or speech	Depression; schizophrenia (negative symptoms)

Rough rule: RHYME AND SPEED = mania · DISCONNECTION AND INVENTED WORDS = schizophrenia · STUCK AND REPETITIVE = organic.

Thought content

Abnormal beliefs

- DELUSIONS — fixed, false, unshakeable
- Overvalued ideas
- Ideas of reference or influence

Preoccupations

- Obsessions and compulsions
- Phobias
- Poverty of thought content

Always ask — risk

- SUICIDAL ideation — intent, plan, means
- HOMICIDAL ideation, or thoughts of harm to others

Screening for suicidal and homicidal ideation is a MANDATORY part of every MSE — never omit it.

"A famous singer is in love with me"

A 28-year-old woman is convinced — with unshakeable certainty and no evidence — that a famous singer she has never met is secretly in love with her.

She believes he sends her coded messages through his songs.

This delusion is called?

A Grandiose delusion

B Erotomanic delusion (de Clérambault)

C Delusion of reference

D Nihilistic delusion

Commit to an answer before the next slide.

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B — erotomanic (de Clérambault syndrome).

A fixed belief that a person, usually of higher status, is in love with them. The "coded messages in songs" is an added delusion of reference.

Delusion subtypes

Subtype	The belief
Persecutory	Being harmed, followed or conspired against — the most common
Grandiose	Inflated worth, power or identity (mania, schizophrenia)
Erotomanic (de Clérambault)	Another, often high-status person is in love with them
Jealous (Othello)	The partner is unfaithful
Nihilistic (Cotard)	The self, body or world does not exist, or is dead or rotting
Somatic	A false belief about the body or health
Reference	Neutral events refer specifically to them
Control / passivity	Thought insertion, withdrawal, broadcasting; being controlled — FIRST-RANK for schizophrenia

The eponyms — de Clérambault, Othello, Cotard — are examined by name, so learn them with their descriptions.

Illusion vs hallucination vs pseudohallucination

Term	External stimulus?	Where perceived / insight
Illusion	YES — a real stimulus misperceived	External; can occur in the healthy, e.g. in fear or dim light
Hallucination	NO external stimulus	Experienced as real and in EXTERNAL space; no insight
Pseudohallucination	NO external stimulus	Perceived in INNER space; insight often retained

Describe each hallucination by: modality, content, second- versus third-person (auditory), and the patient's reaction. Also assess depersonalisation and derealisation, and formication — the sensation of bugs crawling on the skin.

A strange smell before the episode

A 30-year-old man reports brief episodes where he smells burning rubber that no one else notices.

They are accompanied by an intense feeling of déjà vu and a rising sensation in his stomach, and last under a minute.

Most likely cause?

A Schizophrenia

B Temporal-lobe epilepsy

C Alcohol withdrawal

D Cocaine intoxication

Commit to an answer before the next slide.

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ANSWER

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Most likely cause?

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C Alcohol withdrawal

D Cocaine intoxication

B — temporal-lobe epilepsy.

Olfactory hallucinations with déjà vu and an epigastric rising sensation are a classic temporal-lobe seizure aura — an organic, not primary psychiatric, cause.

Hallucination modality → likely cause

Modality	Points toward
Auditory — especially 3rd-person, running commentary	Schizophrenia / primary psychiatric illness
Visual	Delirium, organic disease, or substance-related
Olfactory / gustatory	Temporal-lobe epilepsy (aura), organic
Tactile / formication	Cocaine ("cocaine bugs") or alcohol withdrawal
Hypnagogic (falling asleep) / hypnopompic (waking)	Normal phenomena; also narcolepsy

NON-AUDITORY hallucinations — visual, olfactory, tactile — should raise suspicion of an organic or substance cause until proven otherwise.

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PART 3

Cognition, Judgement & Insight

The bedside cognitive exam · formal testing (MMSE/MoCA) · abstract thinking · judgement · insight.

Consciousness & orientation

LEVEL OF CONSCIOUSNESS — ASSESS THIS FIRST

Alert ▸ **Lethargic** ▸ **Obtunded** ▸ **Stupor** ▸ **Coma**

A clouded or FLUCTUATING level suggests delirium — an organic cause. Interpret the rest of the exam with caution.

ORIENTATION IS LOST IN ORDER

1 · TIME
lost first

▸

2 · PLACE
then place

▸

3 · PERSON
lost last

Disorientation points to an organic or cognitive disorder — not to a primary psychotic illness.

Consciousness and orientation are the foundation — if the patient is not alert and oriented, interpret the rest of the cognitive examination with caution.

Attention & memory

Attention / concentration

- Serial 7s
- Spelling "WORLD" backward
- Digit span
- These test ATTENTION, not registration

Memory

- REGISTRATION / immediate — repeat 3 named objects at once
- RECENT — recall the 3 objects after a few minutes; recent events
- REMOTE — personal and historical long-term memory

Common pitfall: "spell WORLD backward" and serial 7s are ATTENTION tasks. Registration is repeating items immediately; recent memory is recalling them after a delay.

Language, aphasia & visuospatial

Language / aphasia — if brain pathology is suspected

- BROCA'S / expressive — non-fluent, repetition impaired, comprehension INTACT (frontal)
- WERNICKE'S / receptive — fluent but nonsensical, poor comprehension (temporal)
- CONDUCTION — fluent, good comprehension, repetition impaired
- NOMINAL — naming difficulty, e.g. name a pen, a watch

Visuospatial

- Copy a figure — interlocking pentagons, or a clock
- Impairment suggests parietal or organic pathology

MMSE vs. MoCA

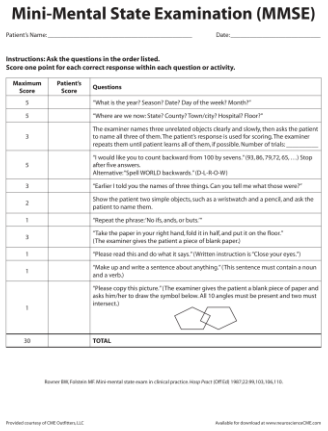
	MMSE	MoCA
Score	Out of 30	Out of 30
Domains	Orientation, registration, attention, recall, language, construction	Adds executive and visuospatial items
Cut-off	≤ ~23–24 suggests cognitive impairment	≤ 25 suggests cognitive impairment (≥ 26 normal)
Sensitivity	WEAK at mild impairment	MORE SENSITIVE for mild cognitive impairment
Access	Now copyright-restricted	Requires brief certification to administer

Both are SCREENING tools, not diagnoses — a low score prompts further assessment and a search for a cause: delirium, dementia, or depression.

MMSE and MoCA — the forms at a glance

Both score out of 30 — but only the MoCA tests executive and visuospatial function.

VERBAL



Mini-Mental State Examination (MMSE)

Patient's Name: _____ Date: _____

Instructions: Ask the questions in the order listed.
Score one point for each correct response within each question or activity.

Maximum Score	Patient's Score	Questions
5		"What is the year? Season? Date? Day of the week? Month?"
5		"Where are we now: State? County? Town/city? Hospital? Floor?"
3		The examiner names three unrelated objects clearly and slowly, then asks the patient to name all three of them. The patient's response is used for scoring. The examiner repeats them until patient learns all of them, if possible. Number of trials: _____
5		"I would like you to count backward from 100 by sevens." (93, 86, 79,72, 65, ...) Stop after five answers. Alternative: "Spell WORLD backwards." (D-L-R-O-W)
3		"Earlier I told you the names of three things. Can you tell me what those were?"
2		Show the patient two simple objects, such as a wristwatch and a pencil, and ask the patient to name them.
1		"Repeat the phrase: No ifs, ands, or buts."
3		"Take the paper in your right hand, fold it in half, and put it on the floor." (The examiner gives the patient a piece of blank paper.)
1		"Please read this and do what it says." (Written instruction is "Close your eyes.")
1		"Make up and write a sentence about anything." (This sentence must contain a noun and a verb.)
1		"Please copy this picture." (The examiner gives the patient a blank piece of paper and asks him/her to draw the symbol below. All 10 angles must be present and two must intersect.)
30		TOTAL

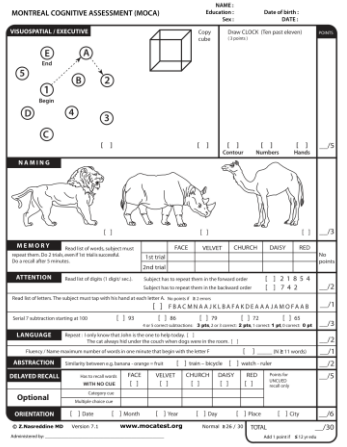
Rovner BW, Folstein MF. Mini-mental state exam in clinical practice. Hosp Pract (Off Ed). 1987;22:99,103,106,110.

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MMSE

Verbal core — orientation, recall, language

+ EXECUTIVE



MONTREAL COGNITIVE ASSESSMENT (MOCA)

NAME : Education : Sex : Date of birth : DATE :

VISUOSPATIAL / EXECUTIVE — Copy cube — Draw CLOCK (Ten past eleven) — Contour — Numbers — Hands — /5

NAMING — /3

MEMORY — Read list of words, subject must repeat them. Do 2 trials, even if 1st trial is successful. Do a recall after 5 minutes. — FACE — VELVET — CHURCH — DAISY — RED — 1st trial — 2nd trial — No points

ATTENTION — Read list of digits (1 digit/ sec.) — Subject has to repeat them in the forward order: [ ] 2 1 8 5 4 — Subject has to repeat them in the backward order: [ ] 7 4 2 — /2

Read list of letters. The subject must tap with his hand at each letter A. No points if 2 (or more) errors — [ ] F B A C M N A A J K L B A F A K D E A A A J A M O F A A B — /1

Serial 7 subtraction starting at 100 — [ ] 93 — [ ] 86 — [ ] 79 — [ ] 72 — [ ] 65 — 4 or 5 correct subtractions: 3 pts, 2 or 3 correct: 2 pts, 1 correct: 1 pt, 0 correct: 0 pt — /3

LANGUAGE — Repeat : I only know that John is the one to help today [ ] — The cat always hid under the couch when dogs were in the room [ ] — /2

Fluency / Name maximum number of words in one minute that begin with the letter F [ ] [ ] [ ] (N ≥ 11 words) — /1

ABSTRACTION — Similarity between e.g. banana - orange = fruit [ ] train – bicycle [ ] watch - ruler — /2

DELAYED RECALL — Has to recall words WITH NO CUE — FACE [ ] — VELVET [ ] — CHURCH [ ] — DAISY [ ] — RED [ ] — Points for UNCUED recall only — /5

Optional — Category cue — Multiple choice cue

ORIENTATION — [ ] Date — [ ] Month — [ ] Year — [ ] Day — [ ] Place — [ ] City — /6

© Z.Nasreddine MD — Version 7.1 — www.mocatest.org — Normal ≥ 26 / 30 — TOTAL — /30 — Add 1 point if ≤ 12 yrs old — Administered by _____

MoCA

Adds trails, cube and clock (executive & visuospatial)

Shown zoomed-out for recognition, not for administration. MMSE: Rovner & Folstein 1987, courtesy CME Outfitters — the instrument is now copyright-restricted (PAR). MoCA v7.1 © Z. Nasreddine MD, mocatest.org — free for educational use with permission. Reproduced here for internal teaching only.

Abstract thinking

What & how

- The ability to handle concepts and draw appropriate inferences
- Similarities and differences — how are a car and a train alike?
- Proverbs — interpret "people in glass houses should not throw stones"

Concrete answers

- A concrete, literal response — "the stones will break the glass" — suggests impaired abstraction
- Seen in schizophrenia, dementia and low IQ
- And NORMALLY in children

Interpret proverb testing with care across cultures and languages — an unfamiliar proverb produces a concrete answer in a perfectly healthy person.

Judgement

What it is

- The capacity to make sound decisions and act appropriately
- Best judged from the patient's RECENT REAL-LIFE BEHAVIOUR

How to assess

- From the history — how have they handled recent situations?
- Hypothetical scenarios are LESS RELIABLE than actual behaviour

Impaired judgement — not just impaired insight — drives much of the risk in mania, psychosis and intoxication.

Insight — awareness of one's illness

INSIGHT IS A SPECTRUM, NOT YES / NO

Full	Partial	Impaired
Recognises the symptoms as illness · able to modify behaviour · cooperates fully with treatment	Acknowledges problems but not as illness · knows others see them as ill · variable behaviour change	Denies illness or problems · no understanding of others' concerns · poor treatment adherence

best adherence and prognosis

poor insight predicts non-adherence and relapse

Describe where the patient sits — don't just tick present or absent.

Record where the patient sits, in every discharge summary — "insight absent" tells the next clinician nothing about how likely he is to stop his medication.

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PART 4

Integration & Review

Key concept distinctions · a worked example · board questions · take-home.

Concept distinctions

Pair	Key difference
Illusion vs hallucination	Illusion = a real stimulus misperceived; hallucination = no stimulus at all
Hallucination vs pseudohallucination	External space and no insight, versus inner space with insight retained
Obsession vs delusion	Obsession is EGO-DYSTONIC, resisted, insight kept; a delusion is held as true with no insight
Overvalued idea vs delusion	Overvalued = strongly held but understandable, and not fixed or bizarre
Mood vs affect	Sustained and subjective (a symptom), versus momentary and observed (a sign)

These five pairs account for a large share of MSE exam questions — know them cold.

A worked example — what a complete MSE reads like

"A dishevelled young man, poor eye contact, restless. Speech rapid and pressured. Mood 'on top of the world'; affect elated and labile.

Thought form shows flight of ideas; content includes grandiose delusions ('I've invented a cure'); no suicidal or homicidal ideation elicited. No hallucinations.

Alert and oriented ×3; attention reduced on serial 7s. Insight impaired; judgement poor."

> **Write the MSE in the same fixed order every time — it makes omissions obvious and lets any reader reconstruct the mental state at a glance. This example reads as an acute manic episode.**

Mr. Y, 24 — "I don't need to be here"

Mr. Y, 24, is brought to the emergency department by his father. You are asked to examine his mental state.

He arrives in a bright orange shirt and mismatched shoes, wearing sunglasses indoors, unshaven, with a strong body odour. He will not sit down, paces the room, and interrupts you repeatedly; he is over-familiar, calling you by your first name.

His speech is loud, rapid and hard to interrupt. He moves from the hospital, to a football match, to a business plan, to his father's driving — each link is understandable but the topic never returns. He says his mood is "excellent, the best of my life", and he laughs, then becomes briefly tearful when his father speaks, then laughs again.

He tells you he has been chosen to reform the national education system and has already emailed the ministry. He denies hearing voices. He has slept about two hours a night for a week. He is fully oriented. He says the only problem is his father, and that he does not need treatment.

Case problem — document his MSE

Questions

- 1 · Describe appearance, behaviour and attitude.
- 2 · Describe speech, and mood versus affect — using both terms correctly.
- 3 · Name the thought-form disturbance, and say how it differs from loosening of associations.
- 4 · Classify his thought content.
- 5 · Comment on judgement and insight.

Model answer

- 1 · Young man, brightly and bizarrely dressed, sunglasses indoors, poor self-care; psychomotor agitation; attitude over-familiar, disinhibited and intrusive
- 2 · Speech increased in rate and volume, pressured, hard to interrupt. MOOD (subjective, sustained) = elated. AFFECT (objective, observed) = labile and expansive
- 3 · FLIGHT OF IDEAS — rapid shifts where the connections are still comprehensible. In loosening of associations the links are not followable, which points to schizophrenia
- 4 · A GRANDIOSE DELUSION — fixed, false, unshakeable, out of keeping with his background, and ACTED UPON
- 5 · Judgement impaired; insight ABSENT — he denies illness and externalises the problem onto his father

Finish by asking what single feature would change the diagnosis toward schizophrenia — the answer being links that cannot be followed, rather than merely rapid.

Board-style questions

#	Question	Answer
1	Fluent but nonsensical speech with poor comprehension — which aphasia?	Wernicke's (receptive) aphasia — temporal lobe
2	Believing a spouse is unfaithful on no evidence?	Othello syndrome — delusional jealousy
3	"Spell WORLD backward" tests which domain?	Attention and concentration — NOT registration
4	Tactile "bugs crawling" hallucinations — two classic causes?	Cocaine ("cocaine bugs") and alcohol withdrawal
5	Difference between an obsession and a delusion?	An obsession is ego-dystonic and resisted, with insight kept; a delusion is held as true without insight

Summary — six things to remember

Golden rules

- The MSE is a SNAPSHOT of the mental state — the psychiatric physical examination
- Separate thought FORM (how) from CONTENT (what)
- Mood = subjective and sustained; affect = observed and momentary

…continued

- Rhyme and speed → mania; disconnection and neologisms → schizophrenia
- NON-AUDITORY hallucinations → suspect an organic or substance cause
- Always assess suicidal and homicidal ideation, insight and judgement

References

Core references

- Kaplan & Sadock's Synopsis of Psychiatry
- Oxford Handbook of Psychiatry
- Sims' Symptoms in the Mind — descriptive psychopathology

Cognitive tools

- Folstein MMSE (1975)
- Nasreddine MoCA (2005)
- DSM-5-TR terminology

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Thank You

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Questions welcome. Sources for every figure are on the references slide.